

5 September 2026

Dr B. K. S. Sastry

Consultant Cardiologist — Heart Failure

CARE Hospitals, Banjara Hills

Hyderabad, Telangana

RE Mr M F Rahaman, M / 63 years, DOB 10 February 1963 — Hyderabad

SUBJECT Atrial cardiomyopathy with HFpEF; paroxysmal atrial fibrillation and flutter; uncontrolled hypertension; untreated severe obstructive sleep apnoea

REQUEST Heart failure specialist review, and reconsideration of two current prescribing decisions

Dear Dr Sastry,

Thank you for your ongoing care of Mr Rahaman. I am a physician and a family member, writing to consolidate his records and to raise four specific points for your consideration.

Summary of established findings

Cardiac MRI (CARE Banjara Hills, 24 July 2026) reports LV volumes as normal (LVEDV 101 ml, 52 ml/m² indexed) with EF 64%, concentric LVH (IVS/PW 1.2 cm), no ventricular late gadolinium enhancement, and biatrial dilatation (LAVI 52 ml/m²) with patchy biatrial free-wall delayed enhancement — reported as atrial cardiomyopathy. Transthoracic echo the preceding day reported EF 50% with grade I diastolic dysfunction; it was performed during tachycardia with ectopy and carried an explicit recommendation to repeat after rate stabilisation. The sleep study reports an AHI of 42.3. The Apollo problem list of 14 August 2026 records atrial fibrillation, systemic hypertension and obstructive sleep apnoea, with CHA₂DS₂-VASc 1 and prior atrial flutter requiring admission in July 2026.

His most recent blood pressure on current therapy, as he reports it, is 150/100 mmHg (pulse pressure 50 mmHg; mean arterial pressure approximately 117 mmHg). His current medication is metoprolol succinate 25 mg each morning, perindopril 2 mg at night, apixaban 5 mg twice daily and atorvastatin 10 mg at night; amlodipine 5 mg was withdrawn at the time of the atrial fibrillation diagnosis.

POINT 1 A haemodynamic observation

His MRI reports a cardiac index of 2.1 L/min/m² and a stroke volume of 64 ml (SVi approximately 33 ml/m²), both below normal, despite a normal ejection fraction and at a scan heart rate of approximately 62 bpm. This low-output physiology — a small, concentrically hypertrophied, poorly compliant ventricle, compounded by loss of the atrial

systolic contribution — appears to me the most likely explanation for his functional decline, which has progressed to breathlessness at under 500 metres in a man who was until recently a competitive cricketer and an active golfer.

I would be grateful for your view on whether this warrants formal assessment with cardiopulmonary exercise testing or a diastolic stress study.

POINT 2 The withdrawn calcium channel blocker

The patient reports that his hypertension was previously well controlled to approximately 120 mmHg systolic on a fixed-dose combination of an ARB 40 mg with amlodipine 5 mg, and that control was lost when the amlodipine was withdrawn following his atrial fibrillation diagnosis.

I would respectfully suggest that the cautions regarding calcium channel blockers in this setting apply to non-dihydropyridines in the context of reduced ejection fraction, and that neither condition is met here: amlodipine is a dihydropyridine without AV nodal effect, and his ejection fraction is 64% on cardiac MRI. The available evidence indicates that second-generation dihydropyridines do not worsen heart failure and were neutral on mortality in trials of patients with LV systolic dysfunction (PRAISE, V-HeFT III), and they are in routine use in anticoagulated AF cohorts.

His haemodynamic picture is one of elevated systemic vascular resistance: normal aortic dimensions on CMR, and a pulse pressure of 50 mmHg that is narrower than would be expected at his age with a systolic of 150, favouring a resistance rather than a conduit-stiffness mechanism. An arterial vasodilator appears mechanistically well matched to it. I would ask that reinstatement be considered, ideally alongside rather than instead of RAS blockade.

POINT 3 Untreated severe obstructive sleep apnoea

An AHI of 42.3 is recorded but no therapy appears to have been initiated. Given the established contribution of severe OSA to resistant hypertension, to atrial fibrillation recurrence and to atrial structural remodelling, I would ask that PAP therapy with formal titration and adherence monitoring be prioritised. This may be the most consequential single intervention available to him.

POINT 4 An important family history, and a drug safety flag

There is confirmed Brugada syndrome in this family: I am myself the proband, with an implantable defibrillator. Two of the patient's brothers have a history of syncopal episodes. Although his resting ECG of 23 July 2026 shows no Brugada pattern (rate 66, PR 157 ms, QRS 90 ms, QTc 434 ms), a normal resting tracing does not exclude a concealed phenotype.

PLEASE AVOID PENDING ELECTROPHYSIOLOGY ASSESSMENT

Flecainide · Propafenone · any class IC agent,
including any “pill in the pocket” strategy

I would suggest a 12-lead ECG with V1–V2 recorded at the second and third intercostal spaces, and would welcome your view on whether cascade genetic testing is warranted. I would also note the recognised overlap in which cardiac sodium-channel loss-of-function produces both Brugada syndrome and familial atrial cardiomyopathy, which may be relevant to the aetiology of his atrial disease.

Further suggestions for your consideration

- Up-titration of perindopril from the current 2 mg toward 4–8 mg, with renal and electrolyte monitoring, before the addition of further agents.
- Addition of spironolactone 25 mg as fourth-line therapy, subject to baseline potassium and renal function — noting its particular rationale in coexisting OSA and atrial fibrosis. If primary aldosteronism is to be excluded, the aldosterone–renin ratio should be drawn first.
- Addition of an SGLT2 inhibitor for the HFpEF diagnosis, irrespective of diabetes status.
- Given diagnosis in July 2026 — within the twelve-month EAST-AFNET 4 window (HR 0.79, 96% CI 0.66–0.94; heart-failure subanalysis HR 0.74, 0.56–0.97, in a stratum where HFpEF was the largest group) — consideration of an early rhythm-control strategy, tempered by the recognition that his biatrial fibrosis burden predicts a lower probability of durable AF ablation success, which should be discussed with him explicitly.
- Separately, his documented atrial flutter carries a Class 1 Level A indication for cavotricuspid isthmus ablation — a procedure whose success does not depend on his atrial fibrosis burden, and which may therefore be the more favourable procedural option.
- Caution against escalating beta-blockade for blood-pressure control, given the low cardiac index and the likelihood of heart-rate-dependent output on exertion.
- **Measurement of his height, which does not appear anywhere in his record.** His MRI indexes six quantities; reconciled against their rounding bounds, these give a body surface area of 1.92–1.93 m², implying a height of 164–171 cm and, at 81 kg, a BMI of approximately 28–30 — on the trial enrolment threshold rather than clearly either side of it. At his weight, a BMI of 30 requires a height at or below 164.3 cm, so a single stadiometer reading settles it. He is of course obese on the Asian-Indian criterion at any of these figures, but the trials used a BMI of 30 and it is that threshold which governs access. Should BMI be confirmed at or above 30, he would meet the BMI entry criterion of the tirzepatide trials in both obstructive sleep apnoea (SURMOUNT-OSA: AHI reduction of 20–24 events/hour versus placebo, with an approved indication) and HFpEF (SUMMIT: cardiovascular death or worsening heart failure, HR 0.62, 95% CI 0.41–0.95) — noting that SURMOUNT-OSA excluded participants with HbA1c ≥ 6.5%, so a documented non-diabetic HbA1c would also be required. The agent would be unusually well matched to his particular combination of problems. I raise this for your consideration rather than as a recommendation, and note that it should complement rather than replace PAP therapy.

Two history items volunteered by the patient that do not appear in his records

He reports a family history of myocardial infarction at approximately age 60 in his grandfather and in an uncle, and separately reports disturbed sleep. The former is relevant to his lipid target on atorvastatin 10 mg; the latter is the symptomatic correlate of the AHI of 42.3 above.

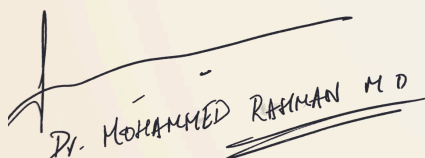
Investigations not currently in his record

No laboratory data of any kind was available to me. I would suggest:

- Potassium and creatinine — essential before any mineralocorticoid antagonist
- HbA1c
- TSH
- Lipid panel
- NT-proBNP, interpreted against an AF-specific threshold
- Ferritin and transferrin saturation
- Aldosterone–renin ratio, drawn before commencing spironolactone
- 24-hour ambulatory blood pressure with nocturnal dipping analysis
- Ambulatory rhythm monitoring, to quantify AF burden and confirm adequacy of rate control on exertion
- Height, by stadiometer

I am glad to provide any of the source imaging or reports, and I am grateful for your time and for your care of him.

Yours sincerely,



Mohammed Faraaz Rahman, M.D.

The Rahman Foundation

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AVAILABLE ON REQUEST

Cardiac MRI report, 24 Jul 2026 (CARE Banjara Hills) · 2D echocardiogram, 23 Jul 2026 · 12-lead ECG, 23 Jul 2026 · polysomnography report · Apollo outpatient summary, 14 Aug 2026 · current and prior prescriptions

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